



Riverbend Care Group

Infection Prevention and Control Policy

Standard and transmission-based precautions, outbreak management and antimicrobial stewardship

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Document control

This document is controlled by the Infection Prevention and Control Lead. Printed copies are uncontrolled and must be checked against the document register before use. Proposed amendments are submitted to the document owner and considered by the Clinical Governance Committee at its next scheduled meeting.

Version history

Version	Date	Author	Summary of change
5.0	2 Feb 2024	J. Achterberg	Consolidated pandemic-era procedures into business as usual.
6.0	15 Oct 2024	J. Achterberg	Added the dedicated infection control practitioner role.
7.0	30 Apr 2026	C. Villanueva	Rewritten against the Strengthened Standards; added stewardship at Part 5.
7.1	26 Aug 2026	C. Villanueva	Revised outbreak thresholds and added the visitor screening requirements at 4.4.

Distribution and accessibility

The current version is published on the provider intranet and is available in hard copy at each nurses' station. Alternative formats, including large print and interpreted summaries, are provided on request to the document owner.

Audience	Method	Frequency
All care and clinical workers	Intranet and induction pack	On publication
Registered and enrolled nurses	Clinical handover briefing	On publication
Consumer Advisory Body	Emailed summary	Quarterly
Contracted and agency staff	Agency portal	Before first shift

Contents

Section	Title
Part 1	Purpose, scope and principles
Part 2	Standard precautions
Part 3	Transmission-based precautions
Part 4	Surveillance and outbreak management
Part 5	Antimicrobial stewardship and immunisation
Part 5A	Water, ventilation and the built environment
Part 6	Governance and accountability for infection prevention and control
Part 7	Workforce competence and training
Part 8	Records, privacy and information management
Part 9	Monitoring, measurement and continuous improvement
Part 10	Risk management
Part 11	Review of this document
Part 12	Communication, engagement and supported decision-making
Part 13	Emergency, contingency and business continuity
Part 14	Subcontracted and external service providers
Part 15	Mapping to the Strengthened Aged Care Quality Standards
Part 16	Assurance, external review and regulatory engagement

Part 1 Purpose, scope and principles

Infection is one of the leading causes of avoidable death and hospital transfer among people living in residential aged care. This document sets out how Riverbend Care Group prevents infection, recognises it early, contains it, and treats it appropriately.

The provider's position is that infection prevention must not be delivered at the cost of the things that make life worth living. Isolation, visitor restriction and cohorting cause real harm through loneliness, deconditioning and distress. Every control applied must be proportionate, time-limited, explained, and lifted as soon as it is no longer required.

1.1 Scope

This document applies to every person who enters a Riverbend service, including workers, agency and contract staff, visiting medical and allied health practitioners, volunteers, contractors and visitors. It covers all sites and the delivery of home services.

1.2 Principles

- Standard precautions apply to every person, at every interaction, at all times.
- Hand hygiene is the single most effective control and is non-negotiable.
- A person is not defined by their colonisation status and must not be treated differently in dignity or engagement.
- Restrictions imposed for infection control require a documented rationale and a review date.
- Antimicrobials are prescribed only where indicated and for the shortest effective duration.
- A worker who is unwell must not attend work, and must never be pressured to do so.

Part 2 Standard precautions

Standard precautions are applied to all people at all times, regardless of known infection status, on the basis that infectious status is frequently unknown.

2.1 Hand hygiene

Hand hygiene is performed at the five moments: before touching a person, before a procedure, after a procedure or body fluid exposure risk, after touching a person, and after touching a person's surroundings. Alcohol-based hand rub is the preferred method unless hands are visibly soiled or the person has a suspected or confirmed infection with a spore-forming organism, in which case soap and water must be used.

Hand hygiene compliance is audited by direct observation each month, with a minimum of 100 observed moments per site. Results are reported by ward and by worker group to the Infection Prevention and Control Committee. Where a site falls below 80 per cent for two consecutive months, the Infection Prevention and Control Lead must present a written recovery plan.

2.2 Personal protective equipment

Task	Minimum PPE	Note
Routine care with no body fluid exposure risk	None beyond hand hygiene	PPE is not a substitute for hand hygiene
Contact with blood or body substances	Gloves and apron or gown	Change between people, always
Risk of splash to the face	Gloves, gown, mask and eye protection	Eye protection means goggles or a face shield, not spectacles
Aerosol-generating procedure	P2/N95 respirator, gown, gloves, eye protection	Fit-checked on every use
Care of a person on droplet precautions	Surgical mask, gloves, apron	Applied before entering the room
Care of a person on airborne precautions	P2/N95 respirator, gown, gloves, eye protection	Fit-tested annually

2.3 Respiratory hygiene and cough etiquette

Tissues, waste bins and hand hygiene products are available at every entrance and in every communal area. A person with respiratory symptoms is offered a mask, supported to perform hand hygiene, and assessed promptly rather than asked to wait in a communal area.

2.4 Safe handling of sharps and waste

Sharps are disposed of at the point of use by the person who used them, into a compliant container filled to no more than three-quarters. Sharps are never recapped, bent or passed hand to hand. A sharps injury is reported immediately, the worker is assessed within one hour, and post-exposure prophylaxis is commenced where indicated without waiting for source testing.

2.5 Environmental cleaning

Cleaning frequencies are set by risk. Frequently touched surfaces in communal areas are cleaned at least twice daily and more often during an outbreak. Cleaning is verified by a documented audit each month, and by fluorescent marker or adenosine triphosphate testing each quarter.

Part 3 Transmission-based precautions

Transmission-based precautions are applied in addition to standard precautions where a person has a suspected or confirmed infection transmitted by contact, droplet or airborne routes.

3.1 Applying precautions

Precautions are commenced on clinical suspicion, not on laboratory confirmation. Waiting for a result before acting is the most common cause of avoidable transmission. The registered nurse in charge may commence precautions without a medical order and must notify the Infection Prevention and Control Lead within 4 hours.

3.2 Minimising the harm of isolation

A person on transmission-based precautions must not be forgotten. Workers must enter the room at least as often as they would otherwise, provide meaningful interaction rather than task-only contact, support video or telephone contact with family, and provide activities in the room. The person and their supporters must be told why precautions apply, what they involve, and when they will be reviewed.

3.3 Review and cessation

Requirement	Timeframe	Responsible
Commence precautions on clinical suspicion	Immediately	Registered nurse in charge
Notify the Infection Prevention and Control Lead	Within 4 hours	Registered nurse in charge
Inform the person and their supporter	Within 4 hours	Registered nurse in charge
Document the rationale and review date	Same shift	Registered nurse in charge
Review whether precautions remain necessary	Daily	Infection Prevention and Control Lead
Cease precautions once criteria are met	Same day the criteria are met	Infection Prevention and Control Lead

3.4 Multi-resistant organisms

A person colonised with a multi-resistant organism is not excluded from communal life, dining or activities. Colonisation is managed through standard precautions and, where clinically indicated, contact precautions during personal care only. Blanket restriction of a colonised person from communal areas is prohibited and will be treated as a non-conformity.

Part 4 Surveillance and outbreak management

Early recognition determines the size of an outbreak. Surveillance is continuous, and the threshold for declaring an outbreak is deliberately low.

4.1 Surveillance

The Infection Prevention and Control Lead maintains surveillance of respiratory infection, gastroenteritis, urinary tract infection, skin and soft tissue infection, multi-resistant organisms and blood-borne virus exposures. Rates are calculated per 1,000 occupied bed days and reported monthly to the committee and quarterly to the Board.

4.2 Outbreak thresholds

- Two or more people with compatible respiratory symptoms in the same area within 72 hours.
- Two or more people with vomiting or diarrhoea in the same area within 48 hours.
- Any single case of influenza, COVID-19, norovirus or a notifiable condition in a resident.
- Any cluster of infection with the same organism in the same area, however small.

4.3 Outbreak response

Action	Timeframe	Responsible
Declare the outbreak and open an outbreak log	Immediately on meeting a threshold	Infection Prevention and Control Lead
Notify the public health unit	Within 24 hours of declaration	Infection Prevention and Control Lead
Notify the Commission where care delivery is affected	Within 24 hours	Chief Executive Officer
Convene the outbreak management team	Within 4 hours of declaration	Chief Executive Officer
Inform residents, supporters and workers	Within 12 hours of declaration	Facility Manager
Commence twice-daily symptom surveillance of all residents	Same day	Registered nurse in charge
Review the need for continuing restrictions	Daily	Outbreak management team
Declare the outbreak over and debrief	Within 5 days of the last case	Infection Prevention and Control Lead

4.4 Visitors during an outbreak

Visiting is not suspended as a default response to an outbreak. Restrictions must be the minimum necessary, must be reviewed daily, and must always permit visits to a person who is dying, a person in significant distress, and a person for whom a visitor provides essential care or communication support. Any restriction lasting more than 7 days requires written approval from the Chief Executive Officer and must be reported to the Board.

4.5 Debrief and learning

Every outbreak is followed by a documented debrief within 10 business days, considering what was recognised late, what worked, what supplies or staffing were short, and what will change. The next scheduled outbreak preparedness exercise falls due by 31 May 2027.

Part 5 Antimicrobial stewardship and immunisation

Antimicrobial resistance develops in the service and travels with the person. Stewardship is therefore a shared clinical responsibility, not a pharmacy function.

5.1 Prescribing

Before an antimicrobial is commenced, a clinical assessment is documented, specimens are collected where practicable, and the indication, intended duration and review date are recorded. Asymptomatic bacteriuria is not treated. A positive urine culture in the absence of symptoms is not an indication for antimicrobial therapy, and treating it causes harm.

5.2 Review

Every antimicrobial is reviewed within 48 to 72 hours against the culture result and the person's response. Courses without a documented stop date are escalated by the nurse in charge. Stewardship data is reported

to the committee quarterly, and the annual stewardship report is due to the Board by 31 July 2027.

5.3 Immunisation

Vaccine	Cohort	Frequency	Target
Influenza	Residents and workers	Annually before 1 May	at least 95% of residents
COVID-19	Residents and workers	As per current ATAGI advice	at least 90% of residents
Pneumococcal	Residents per ATAGI schedule	As scheduled	at least 90% eligible
Herpes zoster	Eligible residents	As scheduled	Offered to all eligible

5.4 Worker health

A worker with fever, respiratory symptoms, vomiting or diarrhoea must not attend work and must not return until 48 hours after symptoms resolve. Managers must not roster, request or permit a symptomatic worker to attend, and must not create financial or rostering pressure to do so. Compliance with this clause is audited each outbreak.

Part 5A Water, ventilation and the built environment

Some infection risks are properties of the building rather than of clinical practice, and are managed through the maintenance program.

5A.1 Water safety

The provider maintains a water safety plan addressing Legionella risk in warm water systems, cooling towers, showers and decorative water features. Temperature monitoring at sentinel outlets occurs monthly, and system testing occurs quarterly. The next scheduled Legionella risk assessment falls due by 28 February 2027.

5A.2 Ventilation

Air handling systems are serviced according to the maintenance schedule, and filters are replaced at the manufacturer's specified interval. Rooms used for the care of people on airborne precautions are assessed for air changes per hour, and the assessment is repeated whenever the room's use changes.

5A.3 Construction and refurbishment

Before any construction, refurbishment or excavation work begins, an infection control risk assessment is completed by the Infection Prevention and Control Lead jointly with the Facilities Manager, addressing dust containment, water disruption, traffic flow and the relocation of vulnerable residents. Work must not commence until the assessment is signed.

Part 6 Governance and accountability for infection prevention and control

The Board of Riverbend Care Group holds ultimate accountability for infection prevention and control and discharges that accountability through the Infection Prevention and Control Committee. The Board receives a standing report at each ordinary meeting covering performance against the indicators set out in Part 9, together with any matter the Infection Prevention and Control Lead considers requires the Board's attention before the next reporting cycle.

The Infection Prevention and Control Committee meets monthly. Its quorum is four members and must include at least one registered nurse and one member independent of the operational management line. Minutes are retained for seven years and are made available to the Aged Care Quality and Safety Commission on request.

6.1 Allocation of responsibilities

Responsibility for the requirements of this document is allocated as set out below. Allocation of a responsibility does not relieve any worker of their own professional and legal obligations.

Role	Responsibility
Board	Receives infection rates and outbreak reports quarterly; approves resourcing for prevention.
Chief Executive Officer	Convenes the outbreak management team; approves visitor restrictions beyond 7 days.
Infection Prevention and Control Lead	Owens this document; maintains surveillance; declares and closes outbreaks; reviews precautions daily.
Facility Manager	Informs residents, supporters and workers; ensures supplies and cleaning frequencies are maintained.
Registered nurses	Commence precautions on suspicion; conduct symptom surveillance; escalate clusters.
All workers	Perform hand hygiene at the five moments; stay away when unwell; report symptoms in residents promptly.
Facilities Manager	Maintains water and ventilation systems; completes construction risk assessments jointly.

6.2 Delegation

The Infection Prevention and Control Lead may delegate an operational function under this document in writing, recording the scope and duration of the delegation in the delegations register. A delegation must not exceed twelve months without review, and the delegate must hold the qualifications and current registration the function requires. The Infection Prevention and Control Lead remains accountable for a delegated function.

6.3 Conflicts of interest

A worker who has a personal, financial or familial interest that could reasonably be perceived to influence a decision made under this document must declare that interest to the Infection Prevention and Control Lead before taking part in the decision. Declared interests are recorded in the conflicts register and reviewed by the Infection Prevention and Control Committee each quarter.

Part 7 Workforce competence and training

No worker may perform a function governed by this document until they have completed the training set out in this Part and been assessed as competent. The Infection Prevention and Control Lead maintains a training matrix mapping every role to its mandatory modules, and the workforce management system prevents rostering a worker to a function for which their competency has lapsed.

Module	Audience	Frequency	Assessment
Hand hygiene and the five moments	All workers	Annually	Direct observation, 100% technique
Donning and doffing PPE	All workers	Annually	Direct observation
P2/N95 respirator fit testing	Clinical workers	Annually	Quantitative fit test
Outbreak recognition and response	Registered nurses and managers	Annually	Scenario assessment
Antimicrobial stewardship	Registered nurses and prescribers	Every two years	Online assessment
Aseptic technique	Registered and enrolled nurses	Annually	Direct observation

7.1 Induction

New workers complete the infection prevention and control component of induction before their first unsupervised shift. Induction covers this document, the escalation pathway in Part 5, and the worker's obligation to report. A worker who has not completed induction may work only under the direct supervision of a competent worker.

7.2 Competency assessment and remediation

Competency is assessed by direct observation against the criteria at Appendix B, not by attendance alone. A worker assessed as not yet competent is placed on a supervised practice plan for a defined period, reassessed at the end of that period, and stood down from the relevant function if the second assessment is not satisfactory.

The Infection Prevention and Control Lead reports training completion rates to the Infection Prevention and Control Committee monthly. Where completion for any mandatory module falls below 95 per cent, the Infection Prevention and Control Lead must provide the Infection Prevention and Control Committee with a written recovery plan at the next meeting.

Part 8 Records, privacy and information management

Records created under this document form part of the care record and are managed in accordance with the provider's Records Management Policy and the Privacy Act 1988 (Cth). Entries must be contemporaneous, factual, legible and attributed to an identified author, and must not be altered after the fact except by a dated and attributed addendum.

8.1 Retention

Record	Minimum retention	Custodian
Surveillance data and infection rates	7 years	Infection Prevention and Control Lead
Outbreak logs and debrief reports	7 years	Infection Prevention and Control Lead
Hand hygiene audit results	3 years	Infection Prevention and Control Lead
Fit testing records	Duration of engagement plus 7 years	Workforce Manager
Water and ventilation testing records	7 years	Facilities Manager

8.2 Access and disclosure

An individual, or their supporter where authorised, may access records held about them on written request. Requests are actioned within 30 days. Personal information is disclosed outside the provider only with consent, or where disclosure is required or authorised by law, and every such disclosure is recorded in the disclosure register.

8.3 Information security

Access to the clinical information system is role-based and individually credentialled. Shared logins are prohibited. Access is revoked on the day a worker's engagement ends, and access logs are reviewed quarterly for anomalous patterns. A suspected data breach is escalated immediately under the Notifiable Data Breaches scheme.

Part 9 Monitoring, measurement and continuous improvement

Performance under this document is measured against the indicators below. The Infection Prevention and Control Lead reports results to the Infection Prevention and Control Committee monthly and to the Board quarterly. An indicator that falls outside its target for two consecutive months triggers a documented improvement action with a named owner and a due date.

Indicator	Target	Source	Frequency
Hand hygiene compliance by direct observation	at least 85%	Monthly audit	Monthly
Resident influenza vaccination coverage	at least 95%	Immunisation register	Annually
Worker influenza vaccination coverage	at least 90%	Immunisation register	Annually
Outbreaks declared within 24 hours of threshold	100%	Outbreak logs	Per outbreak
Antimicrobial courses with a documented stop date	at least 95%	Chart audit	Monthly
Urinary tract infections per 1,000 occupied bed days	Reducing trend	Surveillance data	Monthly

9.1 Internal audit

The Infection Prevention and Control Lead conducts a documented internal audit of compliance with this document at least twice each year, using the tool at Appendix C. Findings are rated as conformity, opportunity for improvement, minor non-conformity or major non-conformity. A major non-conformity must be reported to the Infection Prevention and Control Committee within five business days of identification.

9.2 Continuous improvement register

Improvement actions arising from audits, incidents, complaints, feedback and consumer advisory input are recorded in the continuous improvement register with an owner, a due date and an evidence field. The register is reviewed at each committee meeting and overdue items are escalated to the Board.

9.3 Consumer and workforce input

The provider seeks the views of individuals receiving care, their supporters and the workforce on the operation of this document at least annually, through the consumer experience survey, resident and relative meetings and workforce feedback channels. Results and the provider's response are published in the quarterly quality report.

Part 10 Risk management

Risks to the delivery of the requirements in this document are assessed using the enterprise risk matrix, which rates a risk on the likelihood of occurrence against the consequence for the individual receiving care. Risks rated high or extreme are recorded on the strategic risk register and reviewed by the Board each quarter.

Risk	Rating	Existing controls	Treatment
Outbreak recognised late because clusters are not connected across shifts	High	Twice-daily surveillance; low declaration threshold	Automated cluster alert in the clinical system by 31 March 2027
Workers attend while symptomatic due to financial pressure	High	Paid leave for symptomatic exclusion; manager accountability	Anonymous worker survey on presenteeism by 30 April 2027
Isolation causes deconditioning, delirium and distress	High	Daily review; in-room activity; minimum interaction requirements	Isolation harm audit each outbreak
Blanket visitor restrictions applied disproportionately	High	Daily review; CEO approval beyond 7 days	Board reporting of every restriction exceeding 7 days
Asymptomatic bacteriuria treated with antimicrobials	Medium	Prescribing rule; stewardship review at 48 to 72 hours	Quarterly stewardship audit
Legionella colonisation in warm water systems	Medium	Water safety plan; monthly sentinel monitoring	Legionella risk assessment by 28 February 2027
Construction dust exposure during refurbishment	Medium	Signed infection control risk assessment before work starts	Assessment audited at each project gate

10.1 Dignity of risk

The provider supports each individual to make their own decisions, including decisions that carry risk. Where an individual chooses a course of action a clinician considers unsafe, the discussion, the information given, the individual's decision and any agreed mitigations are documented in the care plan and reviewed at each care plan review. Supporting a person's choice is not a failure of care.

Part 11 Review of this document

This document is reviewed at least every two years, and sooner where a legislative change, a major non-conformity, a serious incident, a coronial finding or a substantive change to practice makes review necessary. The next scheduled review falls due on 1 October 2027.

The review is coordinated by the Infection Prevention and Control Lead and must involve at least one registered nurse, one worker who applies the document in daily practice, and one consumer representative drawn from the Consumer Advisory Body. The reviewed document is approved by the Infection Prevention and Control Committee and, where the change is material, by the Board.

11.1 Related documents

- RB-CLIN-002 Incident Management Policy
- RB-CLIN-007 Medication Management Policy
- RB-OPS-004 Environmental Cleaning Procedure
- RB-CORP-009 Workforce Health and Immunisation Policy
- RB-OPS-011 Water Safety and Legionella Management Plan

11.2 Legislative and standards context

- Aged Care Act 2024 (Cth) and the Aged Care Rules made under it
- Strengthened Aged Care Quality Standards, effective 1 November 2025
- Privacy Act 1988 (Cth) and the Australian Privacy Principles
- Work Health and Safety Act 2011 (Cth) and corresponding state legislation
- Australian Guidelines for the Prevention and Control of Infection in Healthcare

Part 12 Communication, engagement and supported decision-making

Every person receiving care from Riverbend Care Group has the right to be told, in a way they understand, what is happening in their care and why. Information provided under this document must be offered in the person's preferred language and format, and must be repeated or restated where comprehension is uncertain. Reliance on a nod or a signature is not evidence of understanding.

12.1 Interpreters and communication support

An accredited interpreter must be engaged where a person's preferred language is not English and the discussion concerns consent, risk, a change to care, an incident affecting them or a complaint. Family members must not be used as interpreters for these discussions except in an emergency, and where that

occurs the reason is recorded and the discussion repeated with an accredited interpreter within two business days.

12.2 Supported decision-making

The provider presumes that every person has decision-making capacity. Where capacity is in question, the assessment must be decision-specific, time-specific and documented, and must not be inferred from a diagnosis, age or communication difficulty. A person's supporter assists the person to make and communicate their own decision; a substitute decision-maker acts only where the person cannot make the decision even with support.

12.3 Engagement with supporters and representatives

The provider maintains a current record of each person's nominated supporters and representatives, including the scope of any authority held and the instrument granting it. The record is confirmed with the person at each care plan review and whenever the person's circumstances change.

12.4 Culturally safe practice

Care is delivered in a way that is safe for the person's culture, spirituality, gender, sexuality and life experience. For Aboriginal and Torres Strait Islander people, the provider offers connection to community-controlled health services and to family and community, and records the person's wishes about who is involved in decisions about their care.

Part 13 Emergency, contingency and business continuity

The requirements of this document continue to apply during an emergency. Where an emergency makes ordinary compliance impossible, the provider must record what was done instead, why, and for how long, and must return to ordinary practice as soon as the emergency permits.

13.1 Planned system outages

Where the clinical information system is unavailable, workers record required information on the approved paper forms and enter it into the system within 24 hours of the system being restored. The paper record is retained and scanned into the care record; it is not destroyed after entry.

13.2 Surge, outbreak and staffing shortfall

During an infectious disease outbreak or a staffing shortfall, the Infection Prevention and Control Lead may authorise temporary variations to non-statutory requirements of this document. Statutory obligations, including notification timeframes, cannot be varied. Every temporary variation is recorded in the variations log with the authorising officer, the reason, the start date and the review date, and is reported to the next committee meeting.

13.3 Evacuation and relocation

Where a person is relocated in an emergency, the receiving service must be given the person's current care plan, medication chart, advance care directive and supporter contact details before or with the person. A relocation is not complete until a handover has been given to and acknowledged by a named clinician at the receiving service.

Part 14 Subcontracted and external service providers

The provider remains fully accountable for care delivered on its behalf by a subcontractor, agency or external practitioner. Engaging another organisation transfers work; it does not transfer accountability.

14.1 Contractual requirements

- Every service agreement must require compliance with this document by name.
- Every agreement must require the subcontractor to report incidents to the provider immediately, and in any event within four hours of becoming aware.
- Every agreement must permit the provider to audit the subcontractor's compliance on reasonable notice.
- Every agreement must require workers supplied to hold current screening and the qualifications the role requires.
- Every agreement must be reviewed before renewal, and no agreement may be renewed where a material non-conformity remains unresolved.

14.2 Monitoring subcontractor performance

The provider reviews the performance of each subcontractor at least annually against the indicators at Part 9, and more frequently where a concern has been identified. The review considers incidents, complaints, workforce screening currency and audit findings. The next scheduled subcontractor review cycle concludes on 31 March 2027.

14.3 Agency and locum workers

An agency worker may not commence a shift until the provider has sighted evidence of current registration where the role requires it, a current worker screening clearance, and completion of the provider's orientation. The orientation attestation is retained for the duration of the engagement and for seven years afterwards.

Part 15 Mapping to the Strengthened Aged Care Quality Standards

This Part records how the requirements of this document give effect to the Strengthened Aged Care Quality Standards. It is used as the starting point for self-assessment and for evidence gathering before a quality audit.

Standard	Outcome	How this document gives effect to it
Standard 5	Clinical care	Parts 2 and 3 set standard and transmission-based precautions.
Standard 5	Infection prevention and control	Part 4 governs surveillance and outbreak management.
Standard 5	Antimicrobial stewardship	Part 5 requires indication, review and duration limits.
Standard 1	Dignity and choice	3.2 and 4.4 limit the harm of isolation and visitor restriction.
Standard 2	The organisation	4.1 reports infection rates to the committee and the Board.
Standard 3	Care and services	5A requires environmental risks to be assessed and managed.

15.1 Evidence for audit

Evidence supporting each mapped outcome is maintained in the evidence library and indexed against the outcome number. Evidence must be current, dated and attributable, and must demonstrate practice rather than intention: a policy document alone is not evidence that the practice occurs.

15.2 Self-assessment

The Infection Prevention and Control Lead completes a self-assessment against the mapped outcomes at least annually and before any planned quality audit. The self-assessment is endorsed by the Infection Prevention and Control Committee and reported to the Board. The next self-assessment is due by 30 April 2027.

Part 16 Assurance, external review and regulatory engagement

Assurance over this document operates on three lines: the workers and managers who apply it day to day, the Infection Prevention and Control Committee which monitors and challenges performance, and independent review commissioned by the Board. Each line is documented and none substitutes for another.

16.1 Independent review

The Board commissions an independent review of compliance with this document at least every three years, or sooner following a major non-conformity or a serious incident. The reviewer must not have been involved in writing or operating the document. The report goes to the Board unaltered, together with management's response. The next independent review must be completed by 30 September 2027.

16.2 Engagement with the Commission

The provider cooperates fully and promptly with the Aged Care Quality and Safety Commission. Requests for information are acknowledged within one business day and answered within the period specified, or an extension is sought in writing before the period expires. A worker approached directly by the Commission may speak with it freely; the provider must not require a worker to obtain permission first.

16.3 Responding to findings

Where a regulator or reviewer identifies a deficiency, the provider prepares a written response within 14 days setting out what will change, who is accountable, when it will be done and how effectiveness will be measured. Progress is reported to the Board monthly until every action is closed and verified.

16.4 Sanctions and notifiable changes

Where a sanction, a notice to agree, or a notice of non-compliance is issued, the Infection Prevention and Control Lead must notify the Board Chair on the day it is received, and the provider must tell affected individuals and their supporters what it means for their care within five business days.

Appendix A — Definitions

The following definitions apply throughout this document.

Term	Definition
Standard precautions	The minimum infection prevention practices applied to all people at all times regardless of infection status.
Transmission-based precautions	Additional precautions applied for infections spread by contact, droplet or airborne routes.
Outbreak	Two or more linked cases, or any single case of a specified organism, meeting the thresholds at 4.2.
Colonisation	The presence of an organism without signs or symptoms of infection.
Multi-resistant organism	An organism resistant to one or more classes of antimicrobial ordinarily used to treat it.
Aerosol-generating procedure	A procedure that produces airborne particles capable of transmitting infection.
Five moments	The five points at which hand hygiene must be performed during care.

Appendix B — Competency assessment criteria

A worker is assessed as competent when they demonstrate each criterion below by direct observation in the workplace. The assessor records the date, the criterion, the outcome and any remediation required.

Criterion	Evidence sought	Outcome
Performs hand hygiene at the five moments	Observed across 20 consecutive care moments	Competent / Not yet
Dons and doffs PPE without self-contamination	Observed full sequence	Competent / Not yet
Recognises an outbreak threshold	Correctly identifies thresholds in four of five scenarios	Competent / Not yet
Applies precautions on clinical suspicion	States that laboratory confirmation is not required	Competent / Not yet
Describes how to reduce the harm of isolation	Names at least three specific measures	Competent / Not yet
Disposes of sharps correctly	Observed point-of-use disposal without recapping	Competent / Not yet

Appendix C — Internal audit tool

This tool is completed at each internal audit. Each question is answered yes, no or not applicable, with a comment recording the evidence examined. A 'no' answer requires a corrective action with an owner and a due date.

#	Audit question	Evidence examined
1	Is hand hygiene compliance at or above 85 per cent by direct observation?	Audit results
2	Were transmission-based precautions commenced on suspicion rather than confirmation?	Clinical records
3	Was the rationale and review date documented for every person on precautions?	Clinical records
4	Were precautions reviewed daily and ceased promptly once criteria were met?	Precautions log
5	Were outbreaks declared within 24 hours of a threshold being met?	Outbreak logs
6	Were visitor restrictions reviewed daily and approved by the CEO beyond 7 days?	Outbreak logs, CEO approvals
7	Was asymptomatic bacteriuria left untreated?	Charts, culture results
8	Are fit testing records current for all workers requiring a respirator?	Fit testing register
9	Were infection control risk assessments signed before construction work began?	Project records

Appendix I — Abbreviations

The following abbreviations are used in this document and in the records it requires. Abbreviations must not be used in clinical entries where the meaning could be ambiguous.

Abbreviation	Meaning
ACQSC	Aged Care Quality and Safety Commission
ATAGI	Australian Technical Advisory Group on Immunisation
CGC	Clinical Governance Committee
CEO	Chief Executive Officer
EN	Enrolled nurse
IDDSI	International Dysphagia Diet Standardisation Initiative
MDT	Multidisciplinary team
PPE	Personal protective equipment
PRN	Pro re nata, meaning as required
RMMR	Residential Medication Management Review
RN	Registered nurse
SDM	Substitute decision-maker
SIRS	Serious Incident Response Scheme
WHS	Work health and safety

Appendix J — Worker acknowledgement

Every worker to whom this document applies must read it and acknowledge that they have done so. Acknowledgement records that the worker has read the document; it does not replace the competency assessment at Appendix B.

Acknowledgements are recorded in the learning management system. A worker who has not acknowledged the current version within 30 days of its publication is reported to their line manager, and a worker who has not acknowledged it within 60 days may not be rostered to a function this document governs until they do.

Acknowledgement statement

I confirm that I have read and understood this document, that I have had the opportunity to ask questions about it, and that I understand my obligations under it. I understand that where I am unsure how this document applies to a situation, I am expected to ask before acting rather than to assume.

Name	Role	Signature	Date

Completed acknowledgement sheets are returned to the document owner and retained for the life of the version plus seven years.

Appendix H — Implementation plan for the current version

The following milestones apply to the implementation of this version. Milestones are tracked by the document owner and reported to the committee monthly until all are closed.

Milestone	Owner	Due
Publish version 7.1 and brief all clinical workers on the revised thresholds	Document owner	1 Oct 2026
Complete annual fit testing for all clinical workers	Workforce Manager	30 Nov 2026
Commence the isolation harm audit	Infection Prevention and Control Lead	1 Dec 2026
Complete the Legionella risk assessment	Facilities Manager	28 Feb 2027
Deploy the automated cluster alert	Digital Systems Manager	31 Mar 2027
Run the outbreak preparedness exercise and report to the Board	Infection Prevention and Control Lead	31 May 2027

Appendix E — Escalation matrix

This matrix sets out who must be told, and how quickly, when a matter arises under this document. Where two rows could apply, the shorter timeframe governs.

Trigger	Escalate to	Within
Outbreak threshold met	Infection Prevention and Control Lead	Immediately
Single case of influenza, COVID-19 or norovirus	Infection Prevention and Control Lead	Immediately
Sharps or body fluid exposure to a worker	Registered nurse in charge, then occupational health	Within 1 hour
Visitor restriction proposed beyond 7 days	Chief Executive Officer	Before day 7
Suspected Legionella case	Public health unit and Facilities Manager	Immediately
PPE or hand hygiene supply shortage	Facility Manager and Procurement Manager	Same day

Appendix F — Forms and templates

The following forms support this document. Forms are version-controlled and must be obtained from the intranet rather than copied from a local drive.

Form	Purpose	Retained by
RB-F-006a Outbreak log	Records cases, controls, decisions and daily reviews during an outbreak	Infection Prevention and Control Lead
RB-F-006b Precautions rationale record	Records why precautions apply and the review date	Registered nurse in charge
RB-F-006c Hand hygiene audit tool	Records observed moments and technique	Infection Prevention and Control Lead
RB-F-006d Sharps injury report	Records the exposure, assessment and prophylaxis	Workforce Manager
RB-F-006e Construction infection control risk assessment	Records dust, water and traffic controls before work starts	Facilities Manager

Appendix G — Frequently asked questions

These questions arose during consultation on the current version and are reproduced with the answers given.

Question	Answer
Do I need to wait for a swab result before starting precautions?	No. Precautions begin on clinical suspicion. Waiting for confirmation is the most common cause of avoidable transmission.
A resident is colonised with MRSA. Can they attend the dining room?	Yes. Colonisation is managed through standard precautions. Excluding a colonised person from communal life is prohibited and will be treated as a non-conformity.
We are in an outbreak. Do we stop all visiting?	No. Restrictions must be the minimum necessary and reviewed daily. Visits to a person who is dying, in distress, or who relies on a visitor for essential care always continue.
A resident has a positive urine culture but no symptoms. Should we ask for antibiotics?	No. Asymptomatic bacteriuria is not treated. Treating it drives resistance and causes harm without benefit.
I have mild diarrhoea but feel well enough to work. Can I come in?	No. You must stay away until 48 hours after symptoms resolve. No manager may ask or pressure you to attend.

Appendix D — Improvement actions arising from the current review

The following actions were agreed at the most recent review of this document and are tracked in the continuous improvement register until closed.

Ref	Action	Responsible	Due
I1	Introduce an isolation harm audit for every person on precautions beyond 7 days	Infection Prevention and Control Lead	1 Dec 2026
I2	Complete the Legionella risk assessment across all sites	Facilities Manager	28 Feb 2027
I3	Implement an automated cluster alert in the clinical system	Digital Systems Manager	31 Mar 2027
I4	Conduct an anonymous worker survey on attending while unwell	Workforce Manager	30 Apr 2027
I5	Run a full outbreak preparedness exercise at each site	Infection Prevention and Control Lead	31 May 2027
I6	Publish the annual antimicrobial stewardship report	Infection Prevention and Control Committee	31 Jul 2027